



EVALUATING PHYSICIAN ASSISTANT/ASSOCIATE ENGAGEMENT, EXPERIENCE, AND SATISFACTION WITH A LONGITUDINAL ASSESSMENT FOR RECERTIFICATION

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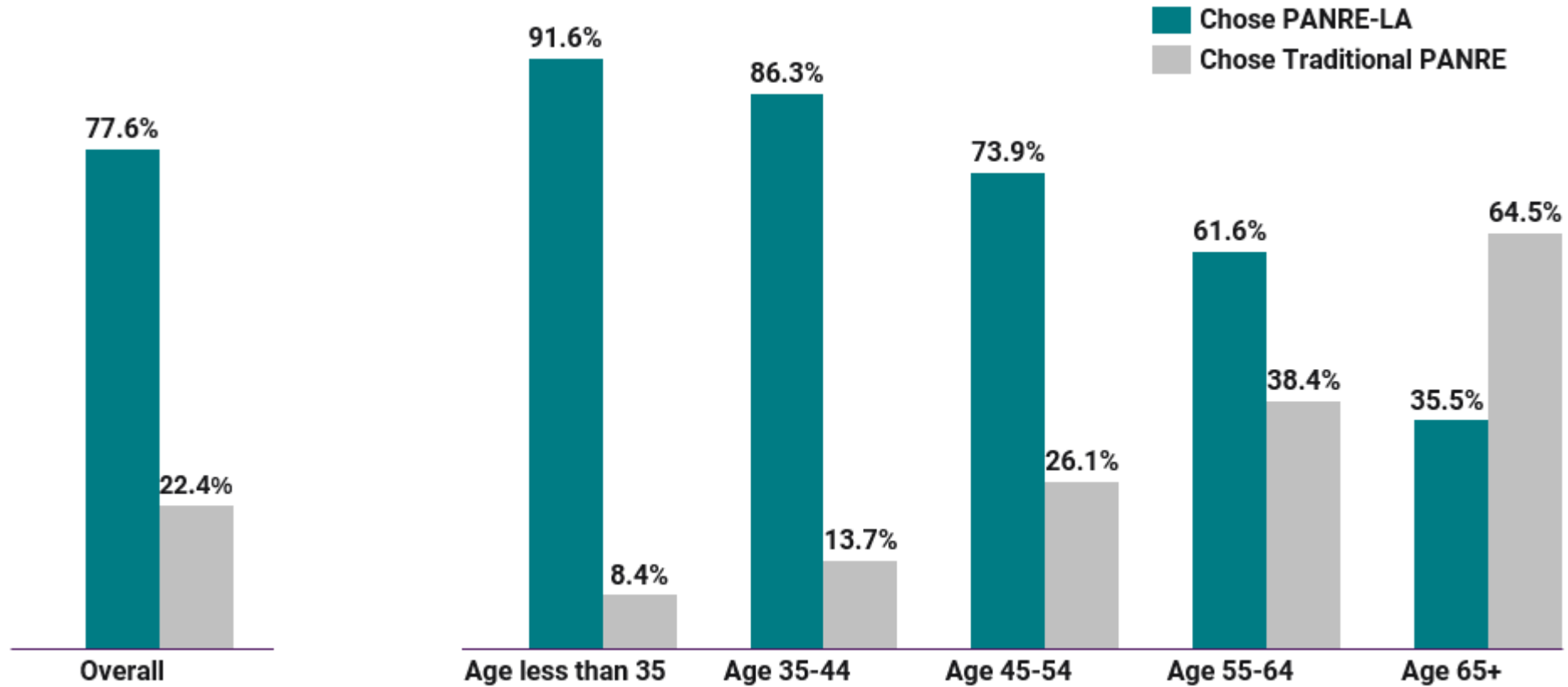
NOTHING TO DISCLOSE

3 Recertification options for PAs

	Traditional PANRE	PANRE-LA
Cadence	240 questions completed in one test-center session	25 questions per quarter; final score based on the best 8 of 12 quarters
Location and resources	Administered at a Pearson VUE test center; external resources are not permitted	Completed on a compatible device; online and print resources are permitted
Feedback and learning	Score typically available within two weeks; primarily summative	Immediate feedback, rationales, and references; repeat topics reinforce identified knowledge gaps

Shared foundation: Both pathways have the same content blueprint and question format, use a scale score to account for question difficulty, and assess core medical knowledge.

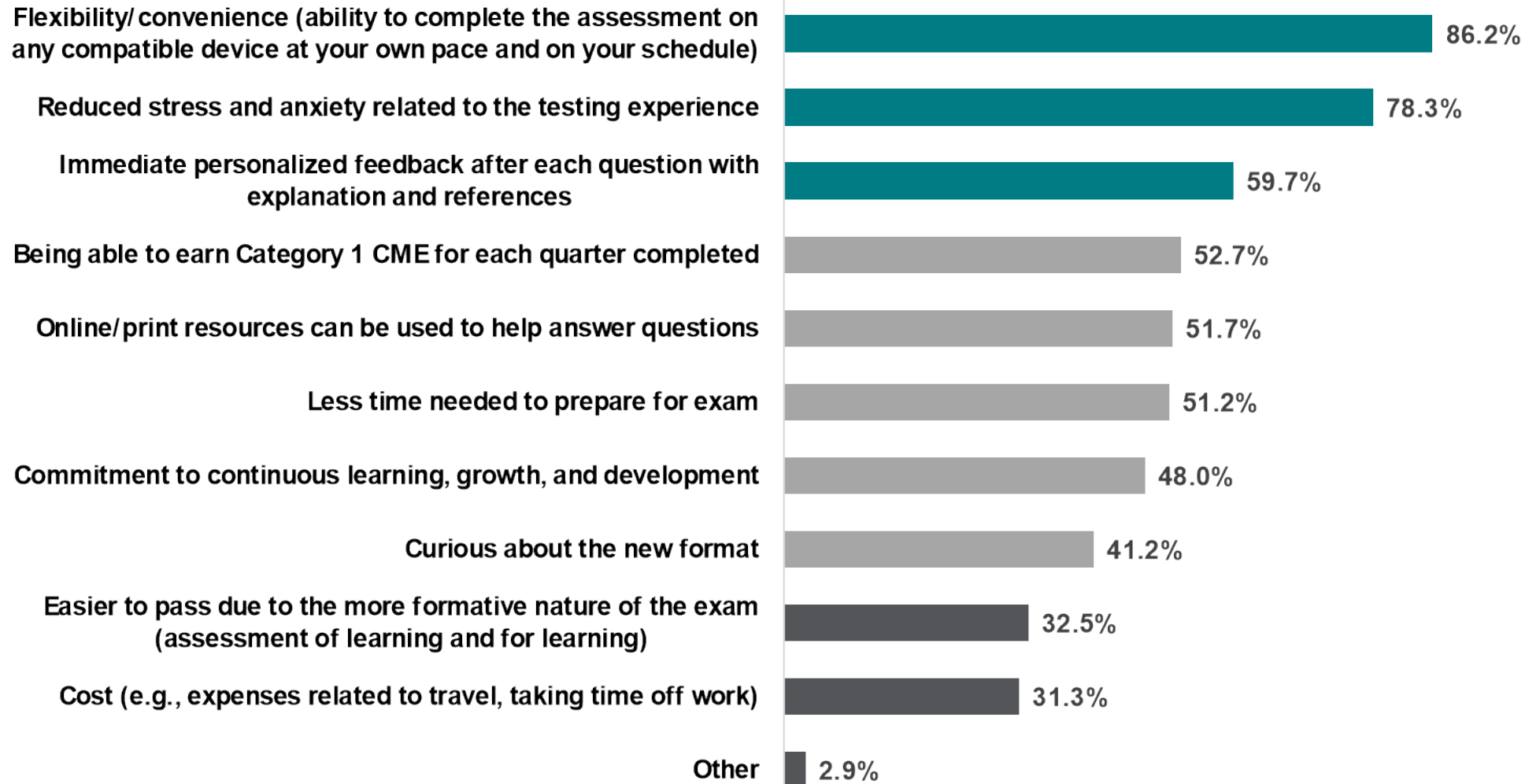
Adoption and age pattern



Cross-sectional analysis of 80,553 PAs due for recertification between 2024 and 2027 using administrative and PA workforce data from the NCCPA¹

Why PAs chose PANRE-LA

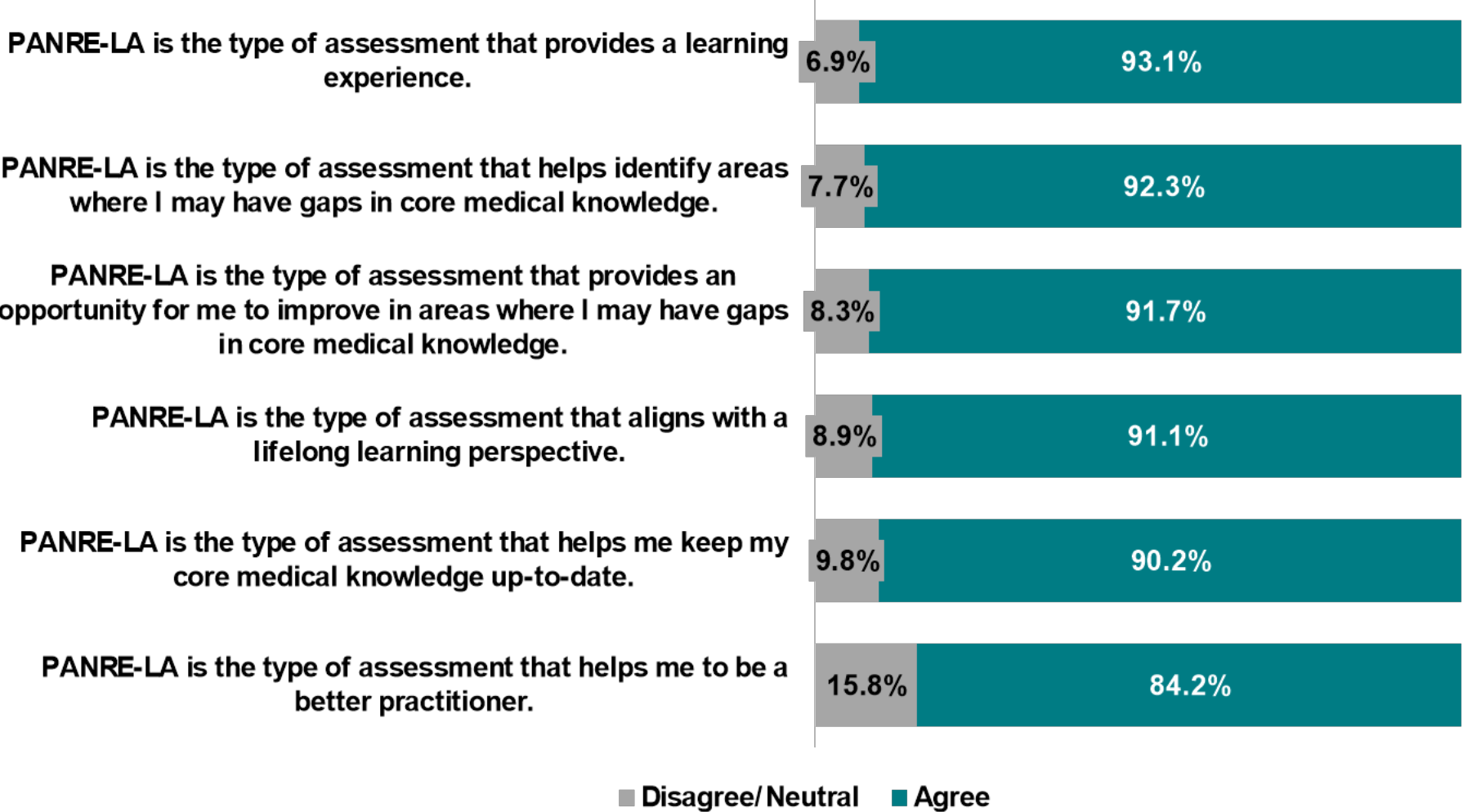
Pooled cross-sectional analysis of 2023–2024 NCCPA survey data from 15,211 PAs after completing their second PANRE-LA quarter²



Note: Multiple responses permitted

Early self-reported learning evidence

Cross-sectional analysis of 2023 NCCPA survey data from 11,896 PAs after completing their second PANRE-LA quarter³



Research gaps and objective



Prior research limitations

- Assessed PA experiences early in PANRE-LA, after only two quarters
- Did not examine later-stage engagement or assessment fatigue
- Did not evaluate overall satisfaction, expectations, comparison with prior point-in-time examinations, or future recertification preferences
- Included limited qualitative feedback

Objective

- To evaluate PA engagement and experience after six PANRE-LA quarters, including satisfaction, fulfillment of expectations, comparison with prior point-in-time examinations, likelihood of recommending PANRE-LA, and future recertification preference

8 Methods

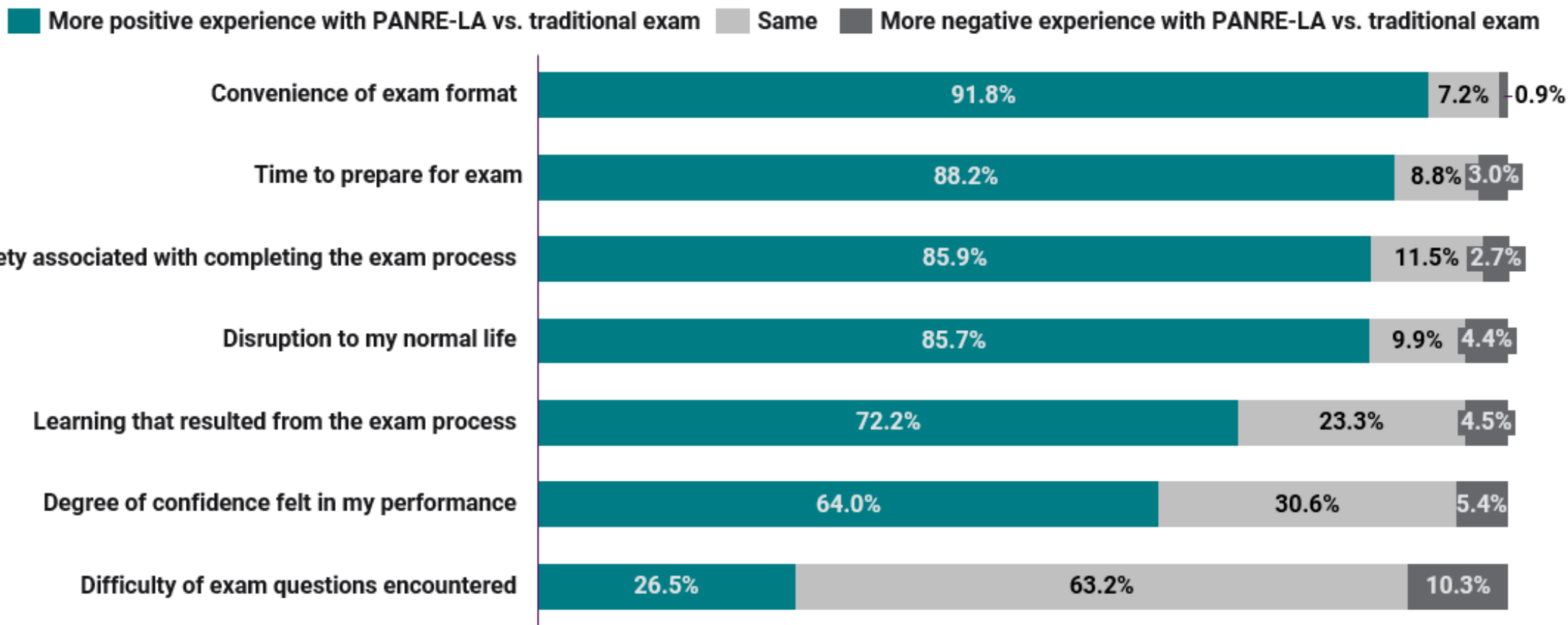
Study Design

- Cross-sectional mixed-methods analysis of a 2024 national survey of PANRE-LA participants (N=8,701; 20.3% response rate)
- Survey administered after participants completed their sixth PANRE-LA quarter
- Demographic and specialty data obtained from the PA Professional Profile and linked to survey responses
- Outcomes included engagement, satisfaction, expectations met, likelihood of recommending PANRE-LA, comparison with prior point-in-time examinations, and future recertification preference
- Open-text responses explored preferences, perceived strengths and limitations, and overall experience

Analyses

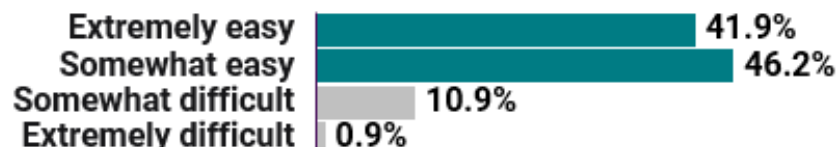
- Quantitative: Descriptive statistics; chi-square or Fisher exact tests with 100,000 Monte Carlo replicates; and multivariable logistic regression with average marginal effects to examine differences by demographic and specialty characteristics
- Qualitative: Thematic analysis of open-text responses

Experience with PANRE-LA vs. traditional point-in-time exams



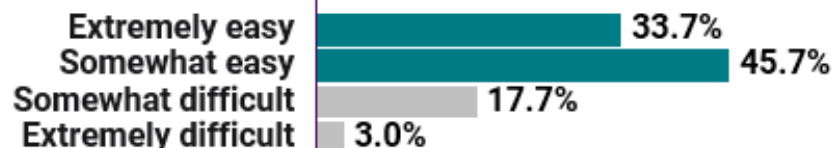
Engagement, expectations, satisfaction, and future recertification

How easy or difficult is it to commit the time to complete 25 questions per quarter?



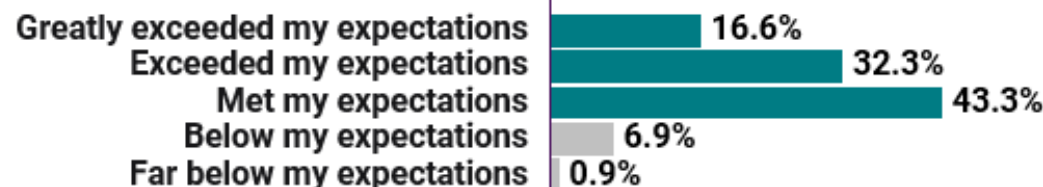
88.2% easy to commit to the time

How easy or difficult is it to stay engaged in a multiple-year assessment program?



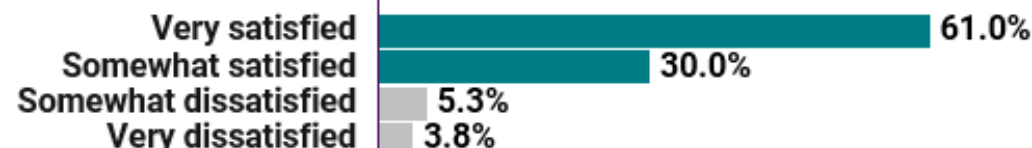
79.4% easy to stay engaged

To what extent did PANRE-LA meet your expectations?



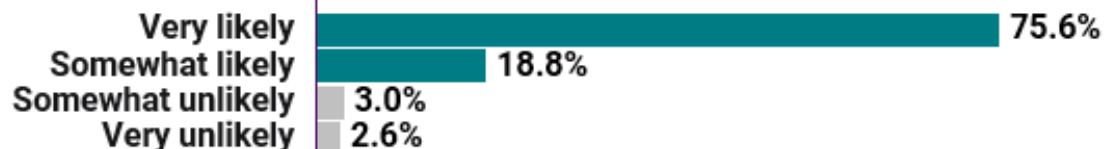
92.2% PANRE-LA met/exceeded expectations

Overall, how satisfied are you with PANRE-LA?



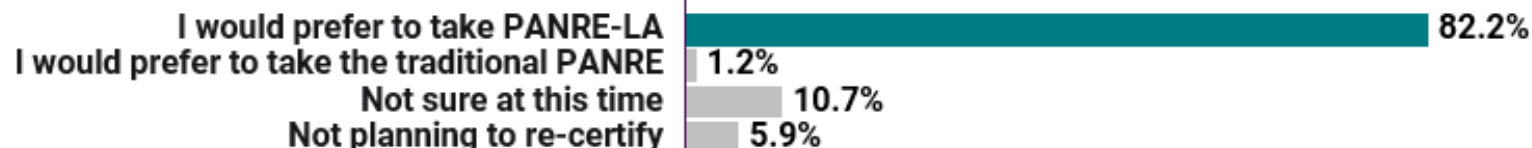
91.0% satisfied

How likely are you to recommend PANRE-LA to other PAs?



94.4% likely to recommend

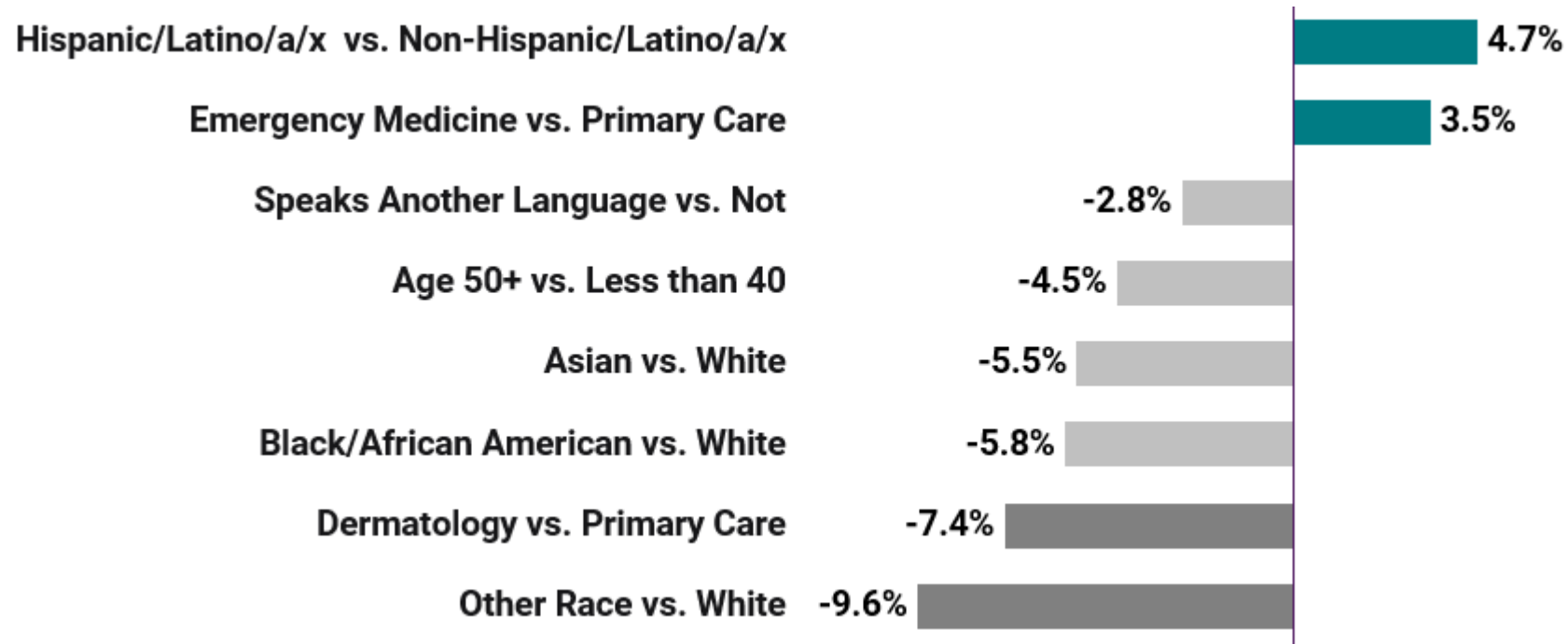
How would you prefer to recertify in the future?



82.2% prefer to take PANRE-LA for the next recertification

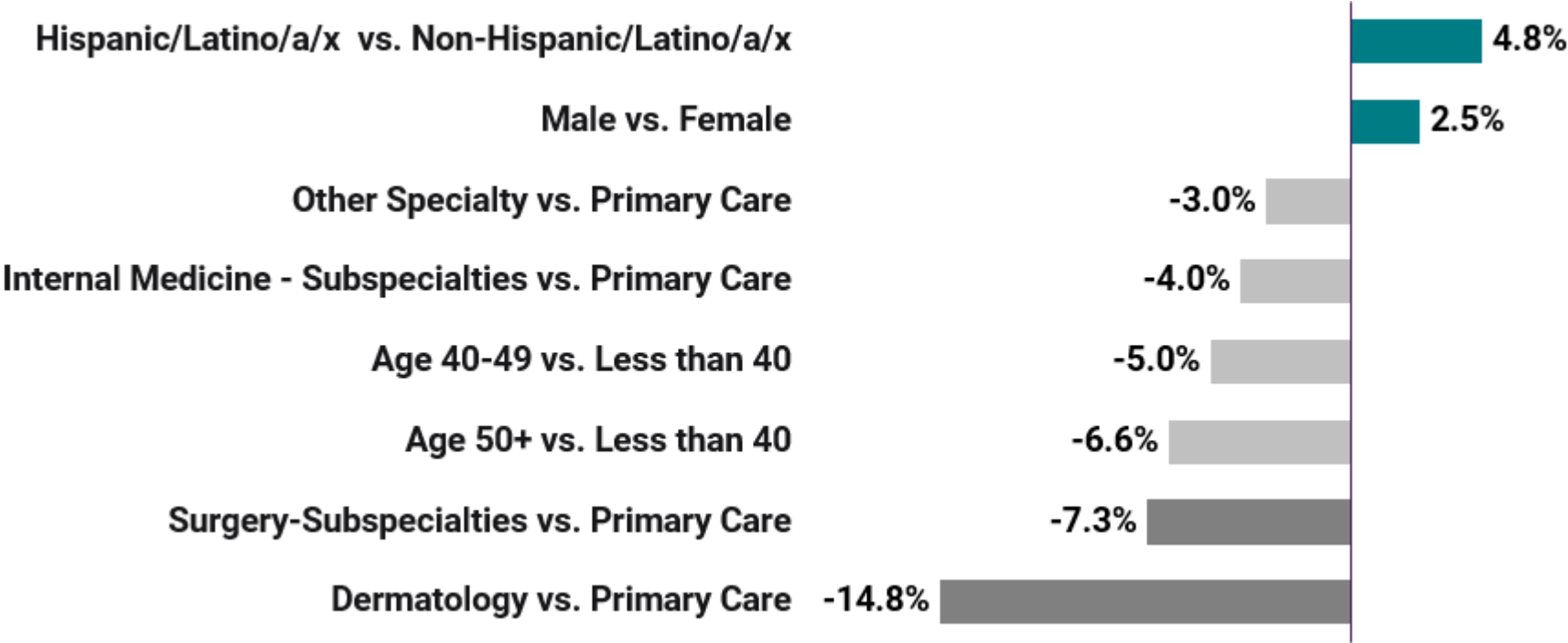
Adjusted associations of indicating it was easy to commit the time to complete 25 questions each quarter

Percentage-point differences from a fully adjusted multivariable logistic regression model. Overall, 88.2% of respondents reported that it was easy to commit the time to complete 25 questions each quarter.



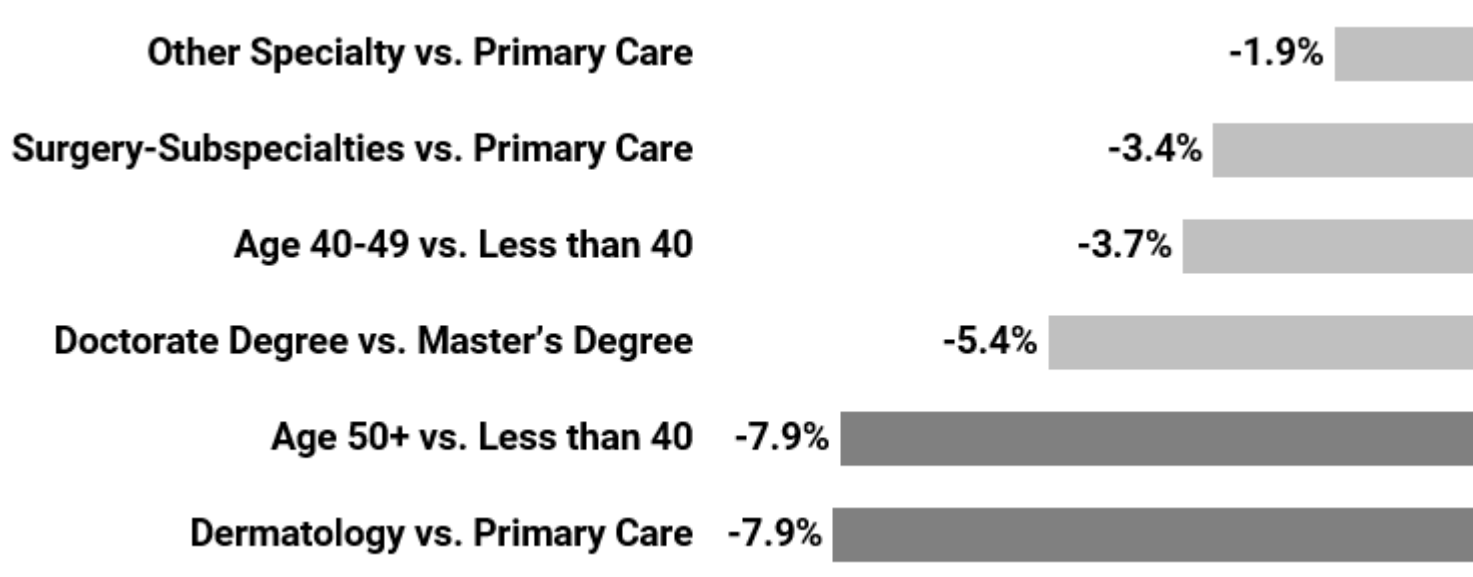
Adjusted associations of indicating it was easy to stay engaged in a multiple-year assessment program

Percentage-point differences from a fully adjusted multivariable logistic regression model. Overall, 79.4% of respondents reported that it was easy to stay engaged in a multi-year assessment program.



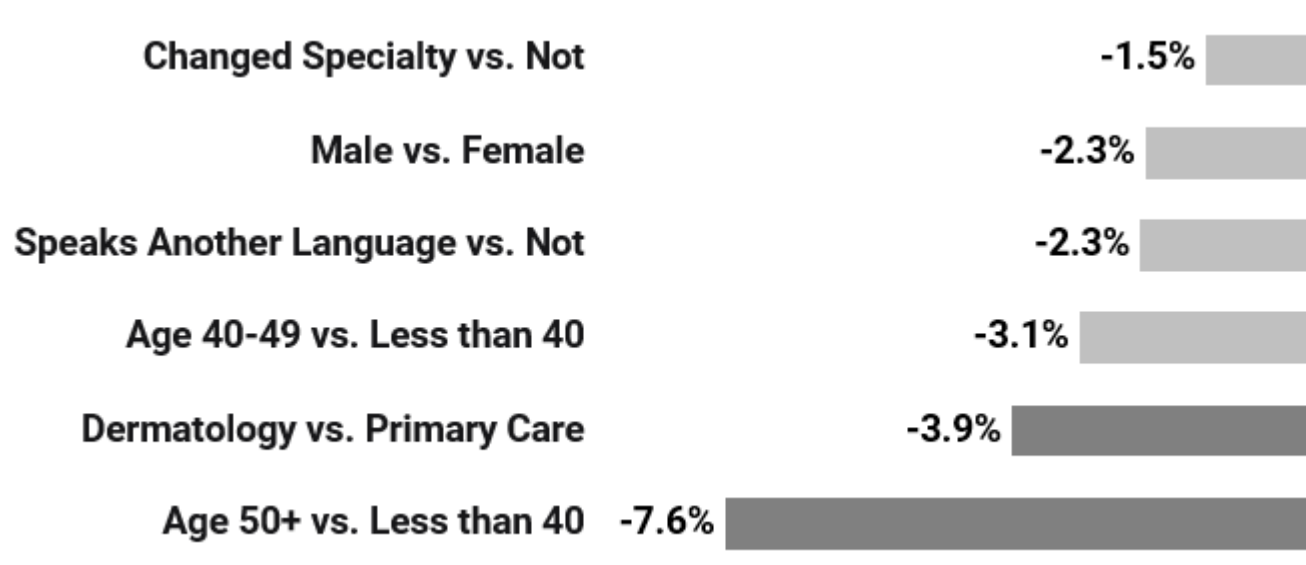
Adjusted associations of indicating PANRE-LA met/exceeded expectations

Percentage-point differences from a fully adjusted multivariable logistic regression model. Overall, 92.2% of respondents reported that PANRE-LA met/exceeded their expectations.



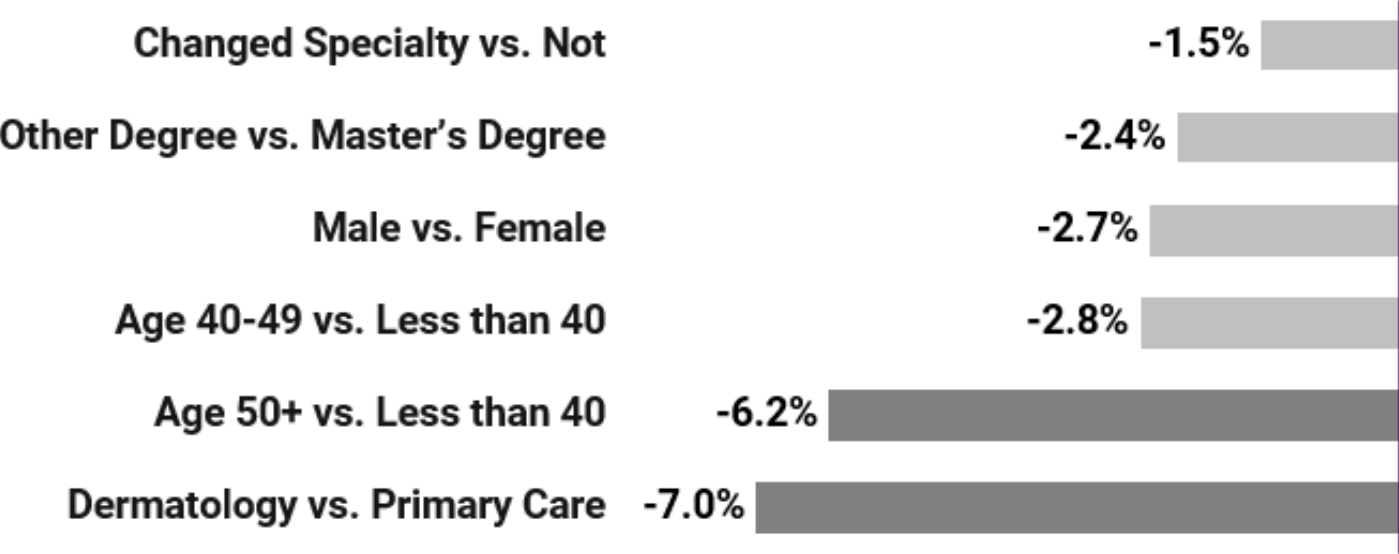
Adjusted associations of overall satisfaction with PANRE-LA

Percentage-point differences from a fully adjusted multivariable logistic regression model. Overall, 91% of respondents reported being satisfied with PANRE-LA.



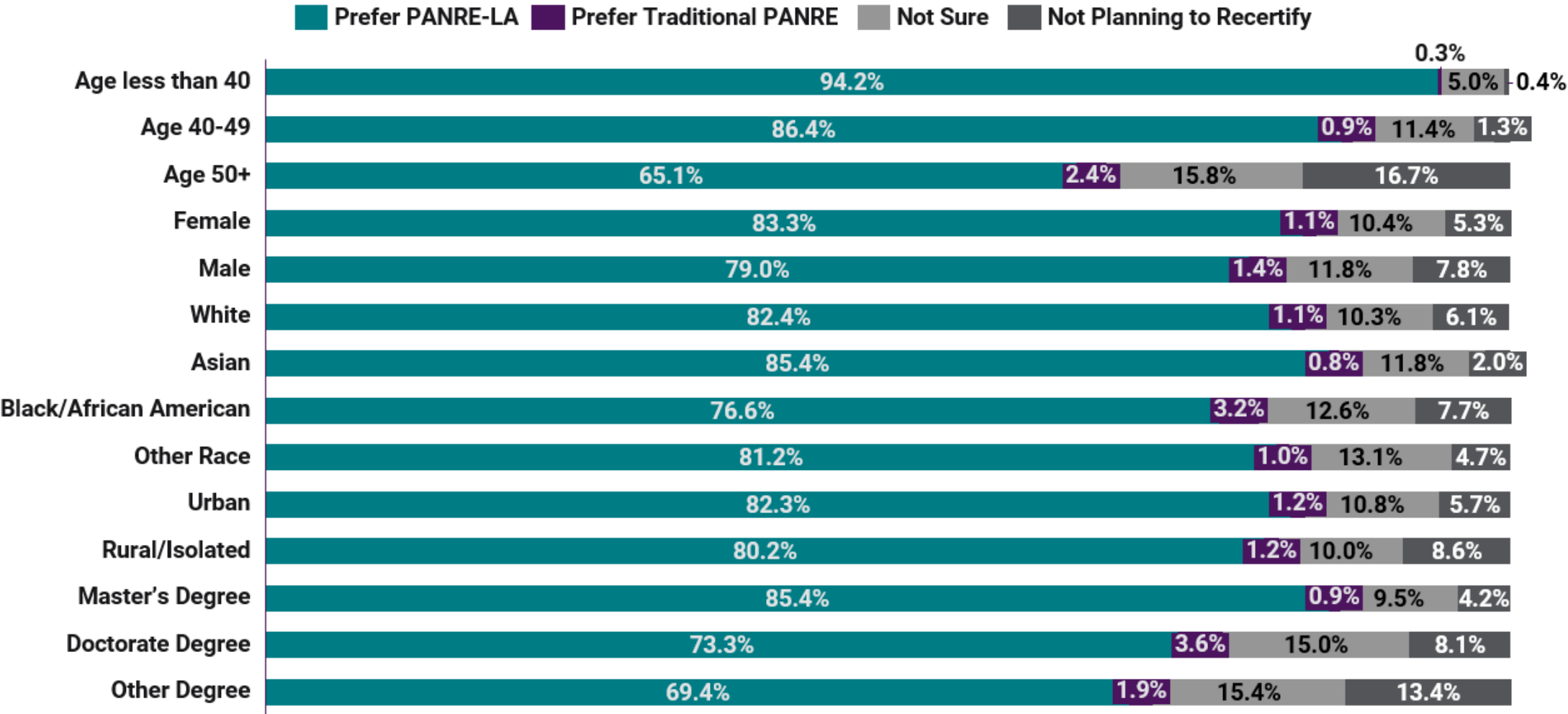
Adjusted associations of recommending PANRE-LA

Adjusted percentage-point differences from a multivariable logistic regression model. Overall, 94.4% were likely to recommend PANRE-LA.



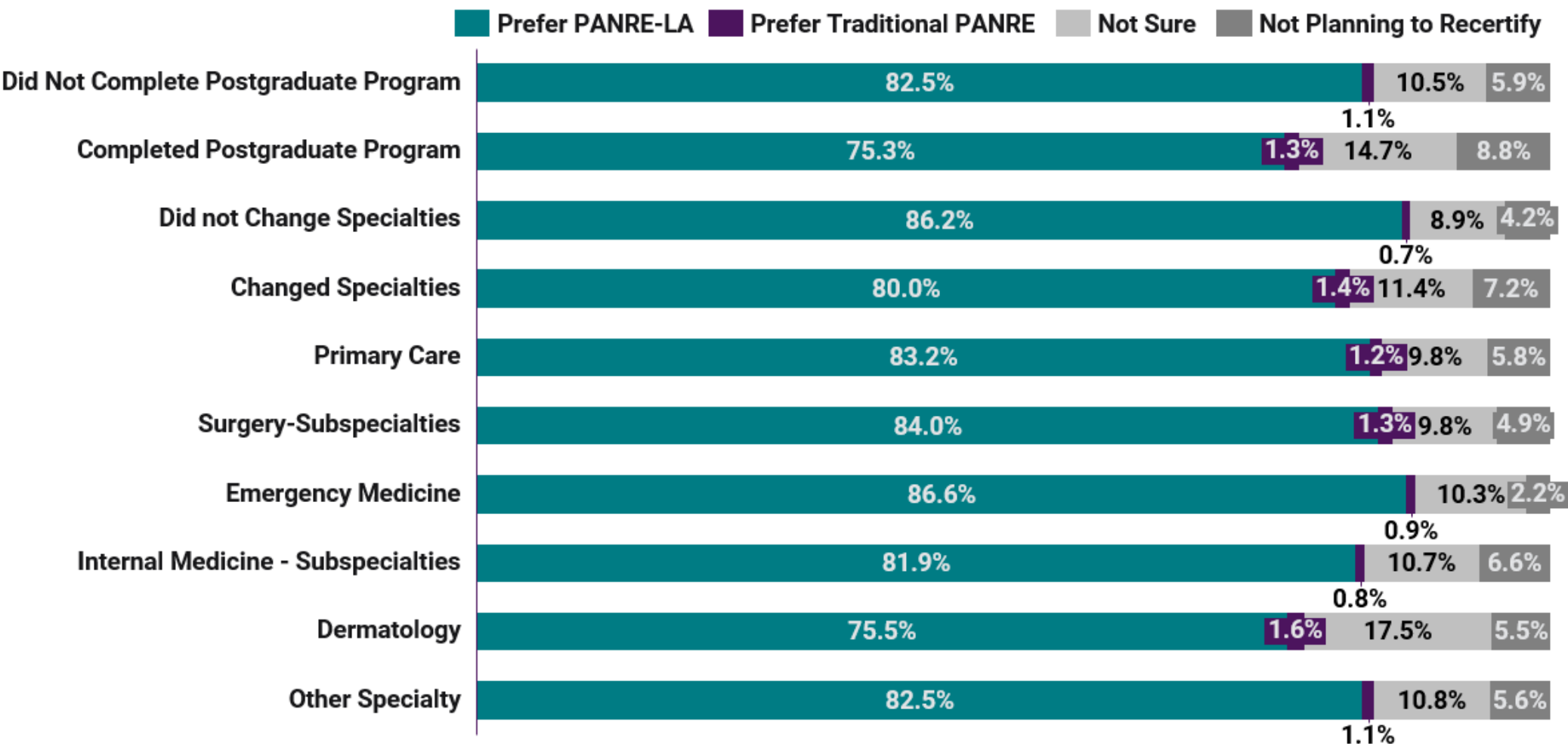
Preferences for future recertification by demographics

Statistically significant results from chi-square analyses or Fisher’s exact test. Overall, 82.2% of respondents prefer PANRE-LA, 1.2% prefer traditional PANRE, 10.7% are unsure, and 5.9% do not plan to recertify.



Preferences for future recertification by workforce characteristics

Statistically significant results from chi-square analyses. Overall, 82.2% of respondents prefer PANRE-LA, 1.2% prefer traditional PANRE, 10.7% are unsure, and 5.9% do not plan to recertify.



Age and specialty differences were accompanied by distinct concerns

Group comparison	Adjusted quantitative findings	Context from participant comments
Dermatology vs. primary care	Easy engagement: -14.8 pp Expectations met/exceeded: -7.9 pp Easy to commit time: -7.4 pp Likely to recommend: -7.0 pp Overall satisfaction: -3.9 pp	General medical content was described as less relevant to daily dermatology practice and increasingly difficult for long-term specialists. Some preferred dermatology-focused recertification or completing all questions in one sitting.
Age ≥50 vs age <40	Expectations met/exceeded: -7.9 pp Overall satisfaction: -7.6 pp Easy engagement: -6.6 pp Likely to recommend: -6.2 pp Easy to commit time: -4.5 pp	Comments described the five-minute limit as stressful and insufficient for effective resource use. Some reported second-guessing correct responses after consulting resources. Suggested changes included additional time or a time bank.

Limitations

Response Rate

- 20.3% (8,701 out of 42,879)

Non-Response Bias

- Respondents vs. non-respondents were slightly older on average (45.7 vs. 43.3), and a slightly higher proportion were female (74.4% vs. 72.6%), White (87.8% vs. 84.3%), and Hispanic/Latino/a/x (6.7% vs. 5.4%)
- Respondents may have held more favorable views

Cross-Sectional Self-Report Data

- Analysis provides a snapshot in time
- Social desirability
- Recall bias

Key Takeaways

Positive perceptions persisted after six quarters

- **Acceptance persisted:** 91.0% were satisfied, 94.4% were likely to recommend PANRE-LA, and 82.2% would choose it for their next recertification. Findings were consistent with physician ratings of different LAs^{4,5}
- **Participants reported a better experience without substantially different perceived difficulty:** PANRE-LA was rated more favorably for convenience, preparation, anxiety, life disruption, and learning; 63.2% rated question difficulty as similar.
- **Sustained engagement:** 88.2% found the quarterly time commitment easy; however, slightly fewer (79.4%) found it easy to remain engaged over multiple years.
- **Differences by career stage and practice context:** PAs aged 50 or older and those in dermatology were less positive across all five outcomes, with comments highlighting time-limited resource use and relevance of generalist content.

Next step: Determine whether sustained PANRE-LA participation translates into objective knowledge gains and changes in clinical practice.



References

1. Kozikowski A, Bruza-Augatis M, Ritsema T, et al. Factors associated with PAs' choice of the traditional PANRE over the PANRE-LA. JAAPA. In press.
2. Kozikowski A, Bruza-Augatis M, Ritsema T, et al. Decoding the choice: what contributes to physician assistants/associates' decision to participate in the longitudinal instead of the traditional assessment for board recertification? JAAPA. In press.
3. Kozikowski A, Goodman J, Dallas A, Jiang Y. Equity and longitudinal assessments: perspectives from physician assistants/associates participating in PANRE-LA. Med Sci Educ. 2025;35(3):1461-1472. doi:10.1007/s40670-025-02329-4
4. Newton WP, Rode K, O'Neill TR, Fain R, Baxley EG, Peterson LE. Family Medicine Certification Longitudinal Assessment after one year. J Am Board Fam Med. 2020;33(2):344-346. doi:10.3122/jabfm.2020.02.190055
5. Leslie LK, Turner AL, Smith AC, Dounoucos V, Olmsted MG, Althouse L. Pediatrician perspectives on feasibility and acceptability of the MOCA-Peds 2017 pilot. Pediatrics. 2019;144(6):e20192303. doi:10.1542/peds.2019-2303